

Care and Support/Support Planning

**AMENDMENT**

Section 12.4, Contingency Planning was updated in June 2025, to include transport arrangements in contingency planning.

[June 13, 2025](#)

1. Using this Procedure

This procedure should be used by anyone working in adult Care and Support carrying out Care and Support or Support Planning following an assessment or review.

2. Introduction and Legal Requirements

2.1 What is a Care and Support Plan?

A Care and Support Plan is a document prepared by the Local Authority which specifies:

- a. The needs identified by the needs assessment;
- b. Which of those needs are eligible (applying the national eligibility criteria); and

- c. Which needs the Local Authority is going to meet and how it is going to meet them.

Legally you must complete a Care and Support Plan when:

- a. The person has eligible needs; and
- b. The Local Authority is going to meet some or all of those eligible needs.

A Care and Support Plan must also be completed if an agreement has been reached to meet ineligible needs following assessment.

2.2 What is a Support Plan?

A Support Plan is the carer's version of a Care and Support Plan, setting out:

- a. All the needs identified by the carer's assessment;
- b. Which of those needs are eligible using the national eligibility criteria; and
- c. Which needs the Local Authority is going to meet and how it is going to meet them.

Legally you must complete a Support Plan when:

- a. The carer has eligible needs; and
- b. The Local Authority is going to meet (or make arrangements to meet) some or all of those eligible needs.

A Support Plan must also be completed if an agreement has been reached to meet ineligible needs following assessment.

2.3 Who to involve in Care and Support/Support Planning

A Care and Support Plan

Whenever you complete a Care and Support Plan you must involve:

- a. The person whose plan it is;
- b. Anyone else that the person has asked you to involve;
- c. Any carer that the person has;
- d. Where the person lacks capacity any other individual who appears to the Local Authority to be interested in the person's welfare.

This means that if the person has capacity you cannot involve anyone else in their Care and Support Plan without their consent to do so.

A Support Plan

Whenever you complete a Support Plan you must involve:

- a. The carer whose plan it is; and
- b. Anyone else that the carer has asked you to involve.

If you feel that anybody else needs to be involved in the Support Plan (for example the person with Care and Support needs or a family member) you can only involve them with the carer's consent to do so.

2.4 The information a Care and Support/Support Plan must contain

The Care Act defines what must be included in any final Care and Support Plan. It is important that you understand what must be included so that any Care and Support/Support Plan that is developed meets the statutory requirements.

Care and Support Plan

See: [The Information that a Care and Support Plan must contain](#) to read the requirements of a Care and Support Plan under the Care Act.

Also see: [Section 18, Recording a Care and Support/Support Plan](#)

Support Plan

See: [The Information that a Support Plan must contain](#) to read the requirements of a Support Plan under the Care Act

Also see: [Section 18, Recording a Care and Support/Support Plan](#)

3. When the Person/Carer intends to Arrange their own Support or Services

3.1 The Purpose of Care and Support/Support Planning

Whenever Care and Support/Support Planning is being carried out at the request of a person/carer who is self-funding, or who intends to use informal or alternative support to meet their needs the process is the same as it would be if the Local Authority were to meet needs.

The purpose of Care and Support/Support planning when a person/carer intends to arrange their own support or services is to:

- a. Use professional support to explore all of the available options to meet needs;
- b. Decide the best way to meet needs from the available options;
- c. Make the best use of their financial resources for the longest time (if they are self funding); and
- d. Decide the best method of arranging services.

Opportunities should still be taken to explore Local Authority options to meet needs alongside the intended informal or alternative options so that the person/carer can make an informed final decision about how to meet their needs through the process.

3.2 After Care and Support/Support Planning

Carers and people living in the community

When the process of Care and Support/Support planning is completed the person/carer will need to decide if they want:

- a. The Local Authority to arrange services;
- b. The Local Authority to arrange and also manage services; or
- c. To make their own arrangements.

Arranging services

If the Local Authority will be arranging services, but not managing them the person/carer can choose whether to maintain the Care and Support/Support Plan or whether to end it, unless they are a person with Care and Support needs and:

- a. They lack capacity and their legal representative decides it would be in their best interests for the Local Authority to maintain the plan;
- b. They lack capacity and the Local Authority decides it would be in their best interests to maintain the plan; or
- c. The person is at risk of abuse or neglect (in this case there **must** be a Care and Support Plan).

If the person/carer wishes to maintain the Care and Support Plan you should:

- a. Submit the draft plan for sign-off;
- b. Agree the final personal budget (to which the Local Authority contribution will be zero and the person's/carer's contribution/contribution from other sources will be the full amount);
- c. Support the person/carer to arrange services/support in the plan where required; and
- d. Arrange for the Care and Support/Support Plan to be reviewed.

Note: If the Local Authority is requested to arrange services by a person/carer who is self funding and has capacity they are legally entitled to charge an administration fee for making these arrangements.

See: [Self Funders](#).

Arranging and managing services

If the Local Authority will be arranging and also managing services the Care and Support/Support Plan **must** be maintained and you should:

- a. Submit the draft plan for sign-off;
- b. Agree the final personal budget and how it will be managed;
- c. Arrange services to be provided by the Local Authority;
- d. Support the person/carer to arrange any other services/support in the plan where required; and
- e. Arrange for the Care and Support Plan to be reviewed.

Making their own arrangements

If the person/carer still intends to arrange all of their own support/services they can choose whether to maintain the Care and Support/Support Plan or whether to end it unless they are a person with Care and Support needs and:

- a. They lack capacity and their legal representative decides it would be in their best interests for the Local Authority to maintain the plan;
- b. They lack capacity and the Local Authority decides it would be in their best interests to maintain the plan; or
- c. The person is at risk of abuse or neglect (in this case there must be a Care and Support Plan).

If the person/carer wishes to maintain the Care and Support/Support Plan you should:

- a. Submit the draft plan for sign-off;
- b. Agree the final personal budget (to which the Local Authority contribution will be zero and the person's contribution/contribution from other sources will be the full amount); and
- c. Arrange for the Care and Support Plan to be reviewed.

If a person with Care and Support needs will be living in a care home and is self-funding

Where the Care and Support Planning process determines that the person's needs are best met in a care home setting (including a nursing home) there is **no** legal obligation for the Local Authority to meet those needs, arrange these services or maintain the Care and Support Plan. However, a decision can still be made using powers under the Care Act to do so if:

- a. The person requests it and the Local Authority agrees;

- b. The person lacks capacity, their legal representative requests it and the Local Authority agrees;
- c. The person lacks capacity and has no legal representative or suitable person to arrange services on their behalf;
- d. The person has complex needs, or needs that are likely to change (if so it may be beneficial for the Local Authority to arrange their service and maintain a statutory review function);
- e. The person's situation is unstable, or at risk of becoming unstable without support from the Local Authority;
- f. The likely need for future Local Authority involvement is high if arrangements are not made on the person's behalf.

If the Local Authority decides to arrange services for a person in a care home they are not permitted to charge an arrangement fee for doing so.

If the Local Authority agrees to maintain the Care and Support Plan and arrange services in a care home you should:

- a. Submit the draft plan for sign-off;
- b. Agree the final personal budget (to which the Local Authority contribution will be zero and the person's contribution/contribution from third parties will be the full amount);
- c. Arrange the care home placement; and
- d. Arrange for the Care and Support Plan to be reviewed.

3.3 Ending the Care and Support/Support Plan

If there is no requirement for the Care and Support/Support Plan to be maintained and the person/carer wishes for it to end you should arrange to do so. However, you must be mindful that the duty to meet eligible needs is not discharged until alternative services are in place.

Upon ending the Care and Support/Support Plan you must:

- a. Provide the person/carer (or their representative) with any information and advice they request (or you feel would be beneficial) about available appropriate services to meet their needs;
- b. Provide them with information and advice about ways they can prevent, reduce or delay needs;
- c. Provide them with any other information that they request or that you feel will be beneficial;

- d. Consider the support the person/carer may need to arrange alternative services (the duty to meet eligible needs is not discharged until alternative services are in place);
- e. Consider the support the person/carer may need to contingency plan;
- f. Explain to them what they should do if the alternative support breaks down in the future;
- g. Agree any monitoring arrangements you deem necessary;
- h. Explain to them what to do if they change their mind about arranging their own services;
- i. Explain to them what they should do if their needs or situation changes; and
- j. In the case of a person with Care and Support needs, continue to carry out any planned Care and Support processes with the carer.

All of the above information should be confirmed in writing.

3.4 Closing the case

You should consider closing the case when:

- a. The Care and Support/Support Plan is ended;
- b. There is no need to monitor the situation; and
- c. The person/carer knows what to do should their circumstances change or they no longer wish to arrange their own services.

4. Delegating Care and Support/Support Planning

Under the Care Act the function of Care and Support/Support planning can be delegated to anyone deemed appropriate who is:

- a. Asking to carry out the function (so long as the person/carer consents or, in the case of a person with Care and Support needs it is deemed in their best interests if they lack capacity);
- b. Willing to carry out the function (when asked to do so by the person/carer or the Local Authority).

Someone can choose to complete the whole of their Care and Support/Support Plan without your support or they can choose to complete a part of it.

Example:

Fred wants to complete the parts of his Care and Support Plan that relate to his Wellbeing outcomes and the things he wants to happen in his life. He also wants to talk with his family about how they can help him to meet some of his eligible needs. However, Fred does not understand the range of service options that are available locally or how best to manage some of the risks associated with his health condition and it is agreed that his social care practitioner will support him with these aspects of the plan.

4.1 Overall Responsibility

The Local Authority is legally responsible for the entire Care and Support/Support Planning process.

Where a Care and Support/Support Planning function is delegated (either in full or part) you will still need to co-ordinate and manage the process to ensure that it is completed in a timely and appropriate way.

Care and Support/Support Plans completed by anyone other than a Local Authority representative still have to be signed off and agreed by the Local Authority.

4.2 Considerations before delegating

Under the Care Act the person/carer is able to complete some or their entire Care and Support/Support Plan without your support if they request to do so and you deem it appropriate. They are able to do this independently, with the support of an advocate or with the support of any other person of their choosing.

Before agreeing to delegate the Care and Support/Support planning function you must provide the person/carer (or their representative) with information about the process and purpose of the function. This will enable them to make an informed choice about carrying it out and the level of

support they may need to be able to do so.

Considerations you will need to make before delegating include:

- a. The views and wishes of the person/carer;
- b. Whether the person/carer (or their representative) understands the process and purpose of Care and Support/Support planning;
- c. The support the person/carer may need to ensure their involvement is ensured (and whether the advocacy duty applies); and
- d. The appropriateness of whoever will be supporting the person/carer (and whether they are agreeable to doing so).

4.3 The process of delegating

Before Care and Support/Support planning can begin the person carrying out the Care and Support/Support planning function must be provided with the information they need to be able to do so. This includes information about:

- a. The Care and Support/Support planning process itself;
- b. The requirements of a Care and Support/Support Plan;
- c. The needs that are to be met through the Care and Support/Support Plan;
- d. The indicative personal budget amount;
- e. Appropriate services available in the local marketplace and the cost of these services;
- f. The prevention services available in the local area and any cost attached to these;
- g. Any general or specific information and advice relevant to or requested by the person/carer (for example around finances or risk); and
- h. Where available the amount they will need to contribute financially to any Care and Support/Support Plan.

If the format is accessible to the person/carer you can provide a blank copy of the Care and Support/Support Plan template used by the Local Authority to record plans as a tool to support them.

If the template is inaccessible, or the person/carer wishes to use their own format they are able to do so (see below for guidance around the format of the plan).

See: [Section 6, Tools and Practice Guidance for Care and Support/Support Planning](#).

When you are satisfied that the person/carer (or their representative) has everything they need to complete the plan you should agree with them:

- a. A timeframe for the plan to be completed; and
- b. How you will monitor progress of the plan; and
- c. What to do if further support is required or the person/carer no longer wishes to complete the plan.

5. The Format of a Care and Support/Support Plan

5.1 Statutory Requirements

The Care Act defines what must be included in any final Care and Support/Support Plan. It is important that you understand what must be included so that any Care and Support/Support Plan that is developed meets the statutory requirements.

See: [The Information that a Care and Support Plan must contain](#) to read the requirements of a Care and Support Plan under the Care Act.

See: [The Information that a Support Plan must contain](#) to read the requirements of a Support Plan under the Care Act.

5.2 The Care and Support/Support Plan format

Because the final Care and Support/Support plan must meet all of the statutory requirements set out in the Care Act, the Local Authority uses a single format template.

If the format of the template is accessible to the person/carer you can use a blank copy as a tool to support the planning process.

5.3 Using a different format

If the format of the template used by the Local Authority is not accessible, or will not ensure maximum involvement of the person/carer it is appropriate to adopt a different Care and Support/Support planning style for some/all of the plan.

Examples include:

- a. Picture or drawing formats;
- b. Video or audio formats;
- c. Alternative written formats.

5.4 Recording when a different format is used

Wherever possible, as long as it meets all the statutory requirements the Care and Support/Support Plan should be recorded in its accessible format.

If the plan format does not meet statutory requirements, or if it is not possible to record and file it as it is (for example the technology used by the Local Authority does not enable electronic filing of audio or video) you should explain this to the person/carer (or their representative) and use the information to complete a final plan in a format that does meet statutory requirements (the Local Authority single format).

The final Care and Support/Support Plan should:

- a. Reflect all of the things from the original plan; and
- b. Explain that a plan in a different format exists and where this can be located.

If it has not been possible to reflect everything in the original plan within the final plan then a copy of the original plan should be attached to the final plan whenever it is provided.

6. Tools and Practice Guidance for Care and Support/Support Planning

6.1 Legal Requirements and Statutory Guidance

Using a tool is not a statutory requirement but can be useful to support conversations during the process of Care and Support/Support planning. However, any tool should:

- a. Facilitate and ensure the person's/carer's involvement;
- b. Support the process of exploring needs from a strengths based approach;
- c. Be flexible and adaptable; and
- d. Be appropriate and proportionate to the situation and needs of the person/carer.

See below for details of the tools that are available for you to use as required.

6.2 Considerations when using tools

Care and Support/Support planning involves having a skilled conversation about:

- a. Wellbeing and outcomes;
- b. Ways to meet eligible needs; and
- c. Risk.

From the assessment (or other process to establish needs) you should have identified how best to communicate with the person/carer and support their engagement in Care and Support processes and any tool that you subsequently use should reflect what you know about the person/carer.

If you do not feel that the tools available to you will be appropriate you should speak to your manager about how they can be adapted.

6.3 Available tools

First and foremost you should have regard for any available practice guidance or good practice examples provided by the Local Authority.

The following are other tools available to you that may enhance any Care and Support/Support planning conversations and accessibility.

tri.x has developed a range of person centred tools that can:

- a. Support a person/carer to think about what matters most to them, now and in the future;
- b. Support a person/carer or family member to think about Wellbeing;
- c. Support a person/carer or family member to think about needs and what a good day/bad day looks like; and
- d. Support a person/carer or family member to think about what is working/not working about a Care and Support/Support Plan and any services or support they receive.

See: [Resources for Person Centred and Strength Based Conversations](#).

Think Local Act Personal have also produced an online tool to support everything from preparing a person for an assessment, to having a skilled conversation and developing a Care and Support Plan through to review. See: [Personalised Care & Support Planning](#).

[Supporting Outcomes-Focused Practice](#) is a supplementary RiPfa website, available without normal subscription. It can support you to develop your skills, knowledge and practice in working with outcomes.

7. Mental Capacity and Engagement Difficulties

7.1 When a person with Care and Support needs lacks capacity

Some people will lack capacity to understand or engage in the Care and Support planning process (verbally or through another means). For example, they may be too unwell to do so or have a significant learning disability. Where this is the case the duty to ensure their involvement still applies.

There are a range of ways that you can ensure the involvement of a person who lacks capacity, including but not limited to:

- a. An appropriate other person or independent advocate to support the person to engage and ensure that they are represented;
- b. Spending time with the person can show you what they enjoy about life and what may be most important to them (this could be a person, a place or something they do with their time);
- c. Consulting with a range of people who know the person before reaching a judgement about Wellbeing, outcomes or how to meet eligible needs (speaking to a family member, a health professional, a paid carer, a college tutor and a day services manager will give a much better picture of what appears to matter most to the person than relying on the views of one person);
- d. Use other available evidence to support you to understand Wellbeing and what matters most (for example ABC charts and other records that show behaviour changes clearly linked to an event, person or place).

All information gathering and sharing should be carried out with regard to the Caldicott Principles, Data Protection legislation and local information sharing policies.

7.2 When a person/carers has substantial difficulty engaging

If a person or carer has substantial difficulty being involved in any Care and Support process you must take all reasonable steps to ensure their involvement.

You must:

- a. Ensure that you have provided information in an accessible way, or that the person/carers has an appropriate person to support them to understand it;
- b. Arrange to carry out Care and Support/Support planning in an appropriate format so that it is accessible. This is likely to be face to face, unless the person's/carers difficulty arises when engaging in face to face communication;
- c. Consider whether the person/carers has an appropriate person to support their involvement and, if not, whether the advocacy duty applies.

See: [Using Independent Advocacy](#), which includes guidance on how to establish whether a person needs an advocate and how to make a referral.

7.3 Managing Disengagement

If the person/carer does not engage in the Care and Support/Support planning process taking place you should:

- a. Establish whether the reason for their disengagement is related to substantial difficulty or mental capacity, and if so ensure they have the right support in place (an advocate or an appropriate other person);
- b. Provide them with information to support them to understand the purpose of a Care and Support/Support plan, the process and the benefits of being involved.

If the person/carer continues to disengage from the process you should establish whether they still wish for the Local Authority to meet the eligible needs identified in the assessment.

If the person/carer no longer wishes for the Local Authority to meet their needs

If the person/carer has changed their mind and no longer wishes for the Local Authority to meet their needs you must:

- a. In the case of a person with Care and Support needs, be satisfied that the person has capacity;
- b. Be satisfied that there is no legal requirement to proceed with Care and Support/Support Planning.

If there is no requirement to proceed with Care and Support/Support Planning you should:

- a. Provide them with the information and advice they request (or you feel would be beneficial) about available appropriate support to meet their needs;
- b. Provide them with information and advice about ways they can prevent, reduce or delay needs for Care and Support/Support;
- c. Provide them with any other information that they request or that you feel will be beneficial;
- d. Consider the support the person/carer may need to access any alternative services;
- e. Consider the support the person/carer may need to contingency plan;
- f. Explain to them what they should do if the informal or alternative support becomes unavailable in the future;

g. Explain to them what they should do if their needs or situation changes; and

h. In the case of a person with Care and Support needs, continue to carry out any planned Care and Support processes with the carer as agreed.

All of the above information should be confirmed in writing.

Where you feel there is a risk that the person/carers will not make appropriate arrangements to meet eligible needs you should make proportionate arrangements with them to monitor the situation, as the duty for the Local Authority to meet eligible needs is not discharged until alternative support/services are in place.

When informal or alternative services are in place and there is no need to monitor the situation you should consider closing their case.

If the person/carers still wants the Local Authority to meet their needs

If the person/carers confirms they still wish for the Local Authority to meet their needs you should:

a. Explain to them that a Care and Support/Support plan is a legal requirement as part of the duty to meet their needs;

b. Establish if there is anyone else that the person/carers wants to represent them in the process; and

c. Explain that you will proceed to complete a Care and Support/Support Plan based on the information gathered in the assessment and local services available.

If the person/carers continues to disengage you should proceed to develop a Care and Support/Support plan based upon the information available from the assessment and the local services available. In the case of a person with Care and Support needs, this should be done in consultation with any carers and, if the person lacks capacity you should involve anyone else that you feel it would be in the person's best interests to involve.

A copy of the draft Care and Support/Support Plan must be provided to the person/carers and they should be invited to review it before it is submitted for sign-off.

Following sign-off a copy of the final plan must be provided to the person/carers in the normal way. If at any point the person/carers changes their mind about engaging (or in the case of a person with Care and Support needs, regains capacity) the plan should be reviewed if this is appropriate and beneficial to the plan and the person's/carers' Wellbeing.

7.4 Refused access to a person/carer

Sometimes another person may obstruct you from carrying out Care and Support/Support planning with a person who has Care and Support needs or a carer with Support needs. You should establish whether:

- a. The person/carer has asked the person to obstruct the process, and if so whether they still wish for the Local Authority to meet their eligible needs;
- b. The person obstructing the process is doing so out of concern for the person/carer (for example would the process cause anxiety);
- c. The person with care and support needs is at risk of abuse or neglect.

Wherever possible you should provide information and advice relating to the Care and Support/Support planning process to the person obstructing it, to support them to understand the benefits and engage in the process.

If the person continues to obstruct the process, and you are not able to establish from the person with Care and Support needs (or carer with Support needs) that this is at their request you must take action to ensure the process is carried out. By obstructing the Care and Support planning process the person is:

- a. Putting the person/carer at risk through lack of support;
- b. Preventing the person/carer from being involved in how their Care and Support/Support needs are met; and
- c. Preventing the Local Authority from fulfilling its duties under the Care Act to meet eligible needs.

A Care and Support/Support planning process in this situation can be carried out based on the information gathered during assessment if this is the only option available. In the case of a person with Care and Support needs, this should be done in consultation with any carer and, if the person lacks capacity you should involve anyone else that you feel it would be in the person's best interests to involve.

The advocacy duty should be considered and it is likely that a referral would be appropriate to ensure that the person's/carer's involvement in the process is ensured.

Opportunities to review the Care and Support/Support Plan must be provided at such time when the person/carer is involved, and all information sharing should give regard to confidentiality.

Depending on the circumstances consideration should also be given to raising an adult safeguarding concern.

See [Safeguarding Adults](#).

8. Using Independent Advocacy

If the need for independent advocacy has not already been established at assessment and you feel that the person with Care and Support needs may lack capacity or a person/carer may have substantial difficulty being involved in Care and Support planning then you must consider whether the duty to make independent advocacy applies and, if so make the necessary arrangements.

See: [Using Independent Advocacy](#), which includes guidance on how to establish whether a person/carer needs an advocate, the different advocates that are available and how to make a referral.

9. Co-ordinating the Process

9.1 The responsibility of co-ordination

As the local authorities representative you have a co-ordinating role regardless of whether you are completing the Care and Support/Support Plan with the person/carer or it has been delegated.

Care and Support/Support planning is a person centred and person led process by law, and you must take steps (or be satisfied that the person delegated is taking steps) to ensure the person's/carer's involvement at all times.

9.2 Preparing the person/carer

To ensure the person's/carer's involvement you must:

- a. Provide the person/carer (or their representative) with accessible information about the purpose of a Care and Support/Support Plan and the process of Care and Support/Support planning before it begins and with sufficient time for them to digest it;

- b. Answer any questions the person/carer (or their representative) has about the process or support them to access appropriate information sources;
- c. Provide information to enable the person/carer to decide whether they wish to complete the Care and Support Plan independently from the Local Authority (see delegating responsibility above);
- d. Provide the person/carer (or their representative) with the indicative personal budget and explain what this is and how it has been decided; and
- e. Consider the need for an independent advocate or appropriate person to ensure involvement.

9.3 Preparing others

In some cases the only person who will need to be involved in the Care and Support/Support Plan is the person/carer. In other cases more people will need to be involved because:

- a. In the case of a person with Care and Support needs, the person has a carer (carers have to be involved);
- b. The person/carer has asked you to involve another person;
- c. The person/carer will be using independent advocacy or an appropriate person; or
- d. A person with Care and Support needs lacks capacity and a decision has been made in their best interests to involve another person.

It is important that everyone who is to be involved in the process is aware of:

- a. The purpose of a Care and Support/Support Plan;
- b. The process of Care and Support/Support planning;
- c. The indicative personal budget; and
- d. The need to ensure the involvement of the person/carer.

9.4 Deciding different roles

Unless the person/carer is completing their own Care and Support/Support Plan independently roles should be discussed and agreed with them before planning starts, and their involvement should always be ensured.

It is important that everyone who is involved in the Care and Support/Support planning process understands the role they have in it.

- a. Finding out information about available local services in the community;
- b. Finding out about how to access a health service that may be beneficial to reduce a need;
- c. Supporting the person/carer to understand the information provided to them;
- d. Co-ordinating a family meeting to talk about informal support that could be provided;
- e. Speaking to a range of care providers;
- f. Exploring risk and contingency;
- g. Supporting the person/carer to think about their most important outcomes and the things they could do to achieve them;
- h. Writing a draft plan; or
- i. Calculating the total cost of draft Care and Support/Support Plan.

When roles are agreed it is important to also agree when different tasks and functions will be completed as this will affect how long the Care and Support/Support Plan takes to develop. It may be useful to prepare a simple written action plan for this purpose that can be shared with everyone involved and used to monitor progress.

tri.x has developed a tool that can be used as required to action plan.

See: [Action Planning Tool](#).

9.5 Deciding the timeframe

The Care Act does not set out a timeframe to complete a Care and Support/Support Plan but it is important that the Care and Support/Support Plan is completed in a timely way so that:

- a. It is able to build on any conversations about ways to meet eligible need that took place during the assessment process in as seamless a way as possible for the person/carer; and
- b. There are no unnecessary delays in the meeting of eligible need.

9.6 Monitoring progress

When a timeframe is agreed it is important that you take steps to monitor progress, particularly where you have limited involvement in developing the plan yourself. The method of monitoring progress should be agreed with the person/carer and should be proportionate and appropriate, maximising the person's/carer's involvement and control over the process.

Some of the factors that should influence the level of monitoring include:

- a. The level of urgency to meet eligible needs;
- b. The number of different people and tasks involved;
- c. The motivation of the person/carer and people involved to complete the plan.

The function of monitoring can be formal or informal, depending on what is agreed with the person/carer and most proportionate. For example monitoring could include:

- a. Face to face via a meeting;
- b. Telephone conversations;
- c. E-mail; or
- d. Text.

Proportionate recording of monitoring activity should always be made.

If a plan is not progressing as agreed you will need to take appropriate action which could include:

- a. Arranging advocacy support for the person/carer if the person supporting them is no longer deemed appropriate;

- b. Arranging a formal meeting to discuss progress and agree a way forward;
- c. Taking responsibility for individual tasks to ensure they are completed; or
- d. Where the Care and Support/Support Plan is delegated, making a judgement that the delegation is no longer appropriate.

All of the above actions and decisions should be made with regard for the person's/carer's views and wishes, and should continue to ensure their involvement and control over the process.

9.7 Urgent Care and Support/Support planning

Sometimes the timeframe within which to Care and Support/Support plan is not conducive to all of the considered planning and preparation as outlined in this procedure. For example, there may always not be the time to research and make the best use of available information or to consult with others before arranging to meet with the person/carer. However, even when the timeframe is short it is of paramount importance that you still:

- a. Take all available steps to ensure the involvement of the person/carer in the process;
- b. Consider consent and mental capacity;
- c. Provide independent advocacy where it is required; and
- d. Prepare the person/carer as much as possible.

If there are urgent needs you may need to arrange interim services to ensure that these are met during the Care and Support/Support planning process. If this is the case see: [Urgent or Interim Support](#).

10. The Skilled Conversation: Outcomes

Need to know

Remember that using a tool to support the process of Care and Support/Support planning can be useful for all involved.

See: [Section 6, Tools and Practice Guidance for Care and Support/Support Planning](#).

10.1 Affirming Wellbeing status

You have a statutory duty to promote individual Wellbeing at all times. During the process of assessment or establishing needs you should have spoken to the person/carer about their Wellbeing and the things that matter most to them (their outcomes).

It is important to affirm during Care and Support/Support planning that the person's/carer's Wellbeing has not altered, and if it has, to ensure that any changes are acknowledged. Wellbeing can change quickly and quite dramatically and you need to understand the person's/carer's sense of Wellbeing now.

See: [Talking about Individual Wellbeing](#).

10.2 Outcomes

Put simply an outcome is anything that the person/carer:

- a. Wants to achieve;
- b. Wants to change; or
- c. Wants to stay the same.

Outcomes are personal and will be different for each person/carer. Outcomes can be related to eligible needs but equally they may not be.

Outcomes should reflect the things that the person/carer wants to achieve and not what other people want to happen.

Outcomes always relate to a sense of Wellbeing, but this is not always in a negative way.

Example:

Sally wants to qualify as a teacher. She is starting University in a few months time and is feeling hopeful. When talking about Wellbeing Sally is clear that the domain 'participating in work, education, training or recreation' domain is most important to her at the moment but in a very positive way.

Example:

Derek wants to be able to use his toilet independently. Not being able to do this is affecting his dignity and when talking about his sense of Wellbeing in that domain he describes how it is very low and starting to affect his Wellbeing in other ways, such as his emotional health.

10.3 The Local Authority's role in meeting outcomes

The Local Authority is not responsible for meeting outcomes, only for taking steps to support them to be met. As long as you can demonstrate that you have taken steps to support the person/carer to meet their outcomes this is all that is required (regardless of whether the outcome is ultimately achieved or not).

Providing services to meet outcomes

Services should not be provided with the sole intention of meeting an outcome. However, you should explore whether any of the services that are to be provided to meet needs could also be used to support the person/carer to achieve outcomes at the same time.

10.4 Understanding outcomes to promote Wellbeing

Because Wellbeing and outcomes are so intrinsically linked you must understand a person's/carer's outcomes in order to fulfil the duty to promote their Wellbeing.

These are the things you need to understand:

- a. What the person's/carer's outcomes are;
- b. What not meeting the outcomes now means for their sense of Wellbeing;
- c. What impact meeting the outcomes would be likely to have on their sense of Wellbeing;
- d. What impact not meeting the outcomes would be likely to have on their sense of Wellbeing.

When establishing the likely impact of meeting or not meeting an outcome on Wellbeing you should use all available evidence to justify your professional opinion. Often, if a person/carer is experiencing a crisis their anxiety levels can affect their ability to rationalise the impact of an outcome on Wellbeing and your role is to:

- a. Support the person/carer to gain insight into their own situation; and
- b. Make a judgement based on evidence.

10.5 Meeting outcomes to promote Wellbeing

When an outcome is likely to have (or is having) an impact on a person's/carer's Wellbeing you should promote Wellbeing by supporting the person to explore and take steps to achieve the outcome. This should be done through a strengths based approach.

You should support the person/carer to:

- a. Think about the steps they may need to take to achieve their outcome (and to break this into manageable chunks if the steps are overwhelming);
- b. Think about the support they may need to take the steps to meet outcomes (including who can support them); and
- c. Think about some of the barriers and challenges to meeting the outcomes (and the things they can do to overcome them).

NHS Choices have an online resource: [5 steps to mental wellbeing](#).

10.6 Unrealistic outcomes

Sometimes a person/carer will have an outcome that you know is outside of their means to achieve (for example something outside of their financial or physical means). Whilst it is important for people to have a goal to work towards, having an outcome that is not realistic will ultimately only have a detrimental impact on Wellbeing because it will likely never be achieved.

You should be sensitive in your approach but you must endeavour to support the person/carer to realise that the outcome they have may not be achievable. You should begin by following the 3 steps outlined in the previous section.

You could also encourage the person/carer to break the outcome down into smaller, more realistic outcomes that may achieve the same positive effect on their Wellbeing.

Example:

Craig's initial outcome is to visit India on holiday and he tells his social care practitioner that not being able to do so is having a negative impact on his Wellbeing. She knows that Craig has a serious health condition and when she seeks advice from his healthcare professional she is told that long distance travel would not be recommended and would pose serious risk. This makes his outcome unrealistic.

She talks to Craig in a positive way about why he wants to go to India and what particular things he hopes to gain from doing so. Craig talks about a liking for Indian food, a lack of local restaurants and a wish to go on holiday somewhere warm. The social care practitioner establishes that if Craig were able to access more Indian restaurants he may not feel the need to go to India, but would still want to go somewhere warm on a short holiday.

Craig has an independent advocate who supports him to explore Indian restaurants in the next town and suitable transport links to get there and back safely. They also purchase some authentic Indian recipe books and prepare a few meals at home. Craig enjoyed this so much that he now holds regular Indian nights for his brother.

Craig no longer has an outcome to visit India but is now being supported to explore places for a warmer break close to the UK and his healthcare professional has given him information about countries he could travel to where the risks associated with his health condition are reduced. He says that the changes he has made have had a positive effect on his sense of Wellbeing.

10.7 Monitoring whether outcomes have been met

It is important to monitor whether outcomes have been met whenever a Care and Support/Support Plan is reviewed and whenever needs are reassessed.

However, because of the potential impact on the person's/carer's Wellbeing you may decide that there is a need to monitor outcomes more frequently.

If this is the case, see [Section 12, Deciding how the Plan will be Reviewed and Monitored](#).

11. The Skilled Conversation: Ways to Meet Needs

Need to know

Remember that using a tool to support the process of Care and Support/Support planning can be useful for all involved.

See: [Section 6, Tools and Practice Guidance for Care and Support/Support Planning](#).

There is a range of ways that the Local Authority can meet eligible needs under the Care Act, ranging from providing information and advice to professional support to the provision of a service. See: [How Needs Can Be Met](#), part of the Care Act 2014.

11.1 Strengths Based Approach

When talking about meeting needs the starting premise should always be that the solution can be found somewhere from within the person/carer, their existing networks of support or their local community. Whenever available these solutions are always the most proportionate, the most likely to sustain a person's/carer's independence and the most effective in avoiding the need for formal intervention. They support the person/carer to take control of their own life and promote resilience.

See: [A Strengths Based Approach](#).

11.2 Information and Advice

The duty to provide good information and advice and to prevent, delay or reduce needs applies at all times. Good information and advice supports the person/carer to make an informed choice about the best way to meet their eligible needs.

See: [Providing Information and Advice](#) for information about the duty to provide good information and advice, including the duty to make sure that information and advice is accessible to the person/carer receiving it.

As well as providing any information and advice specifically requested by the person/carer you should provide the following information and advice as part of the Care and Support/Support planning process:

- a. Information about appropriate available types of service to meet eligible needs and the cost of these;
- b. Advice about which ways of meeting needs may or may not be appropriate, based on evidence and your own professional knowledge;
- c. Information and Advice about ways to prevent, delay or reduce needs (including signposting or exploring referrals to a prevention service);
- d. Information about how services are arranged (including where relevant how services are arranged for self funders);
- e. Information about the Sign-off process of the Care and Support/Support Plan;
- f. Information about how a final personal budget will be decided; and
- g. Information about options to manage the personal budget, including Direct Payments.

You should also ensure that you have given the person/carer and anyone else involved time to ask questions or seek clarity about the process and available options to meet needs.

11.3 Choosing which service to use

'Meeting needs' is an important concept under the Care Act and intentionally moves away from the previous terminology of 'providing services' for a reason. The Act expects you to take a diverse approach to the meeting of needs, which includes being able to support the person/carer to also take a diverse approach to the meeting of needs.

When you have decided with the person/carer the type of service to best meet needs (for example informal care, a community based service, home care, residential care) you will need to explore available options.

It is important that the person's/carer's involvement in this decision is ensured. To do this you may need to consider a range of ways to explore options, depending on the person's/carer's needs and circumstances.

Examples could include:

- a. Providing accessible information about different services;
- b. Allowing time for the person/carer (or their representative) to research the providers online or through making direct contact with them;
- c. The use of photographs to support a person to visualise a placement;
- d. Arranging for the provider to visit the person at home;
- e. Arranging for the person to visit a placement and meet staff and people who live there;
- f. Testimonies of people already using a service.

11.4 Managing different views about the best way to meet need

Exploring how to meet needs from a strengths based approach can involve having some challenging conversations, particularly when the person (or their representative)/carer has traditional service-led ideas about the meeting of needs. However it is your role to support the person/carer to explore the full range of options available to them and to make an informed decision about which would best meet needs.

You may also find the tool [Six Tips to Help You Have Difficult Conversations](#) useful.

You should make effective use of supervision to explore and develop skills that will support you to have difficult and sensitive conversations with people in a positive way.

Where differences in views about meeting needs persist you are responsible for ensuring that the Care and Support/Support Plan that is submitted for sign-off:

- a. Meets all of the statutory requirements;
- b. Has explored all of the available options; and
- c. Reflects the most appropriate option to meet the person's/carer's needs.

You are also responsible for ensuring that all reasonable steps have been taken to agree with the person/carer the best way to meet their needs. As long as you have done this then, where disagreement remains you are ultimately responsible for making the decision about how best to do so.

Where ongoing disagreement persists you should:

- a. Seek the support and advice of your line manager as required; and
- b. Make proportionate records of any conversations you have had to try and resolve the differences.

You must also make the person/carer (or their representative) aware of their right to complain about the decision that has been made.

12. The Skilled Conversation: Risk and Contingency

12.1 Understanding what risk is

Risk is broadly defined as 'the probability that an event will occur with beneficial or harmful consequences'.

The aim of any conversation about risk is to maximise the benefits and reduce the likelihood of harm.

12.2 Talking about risks to the person/carer

During the Care and Support/Support planning process there will be a need to talk about risks to the person/carer:

- a. From the impact of their assessed needs (for example from falls or medication); or
- b. From the outcomes they want to achieve (where there is an element of risk taking required to achieve them).

See: [Talking about Risk](#).

12.3 Talking about risks to the Plan

As part of the Care and Support/Support planning process you must explore potential risks to the plan itself. Not doing so could place the person/carer at significant harm should an element of the plan break down or fail to be delivered.

The kind of risks to the plan will vary depending on each person's/carer's needs and circumstances and any conversation that you have should be proportionate and appropriate.

Examples could include:

- a. Risks that informal care will not be sustainable;
- b. Risks that needs will not be met if a service provider is temporarily unavailable;
- c. Risks that the support or services in the plan will not be as effective at meeting needs or supporting the person/carer to achieve outcomes as intended;
- d. Risks that a Direct Payment will not be managed appropriately.

The person (or their representative)/carer should be involved in any conversation about risk and how to manage risk. In the case of a person with Care and Support needs, any carer should also be involved.

You need to understand:

- a. What the risk is;
- b. What the impact of the risk would be to Wellbeing of the person; and

- c. How likely the risk is to occur (based upon available evidence).

Depending on the level of risk a formal risk assessment may need to be carried out. See: [Risk Assessment](#).

12.4 Contingency Planning

When risks have been identified a contingency plan will need to be agreed with the person/carer. A contingency plan is a proactive agreement about the steps that will be taken to mitigate and respond to the identified risk.

A contingency plan should include:

- a. Details of the risk;
- b. The likely harm to the person/carer if the risk occurs;
- c. The proactive measures that will be taken to mitigate the risk; and
- d. The reactive measures to any risk that occurs.

The conversation about the contingency plan should be from a strengths perspective. If the person/carer is able to identify their own solutions and measures to mitigate and respond to risk they will have much greater control about what happens when risk occurs and how best to resolve issues in the best way for them.

Examples of proactive measures that can be taken to mitigate risks could include:

- a. Being part of a community group to build networks of local support;
- b. Training for a carer to support them to manage the person's needs or carry out safe manual handling;
- c. Clear transport arrangements if provided in the Care and Support Plan. This should include a plan for unforeseen circumstances such as access to a mobile phone, what to do and who to ring if there is an issue or the transport does not arrive;
- d. Access to a specialist health service to maintain and promote good health;
- e. A sitting service for a carer to enable regular breaks;

- f. Respite away from home to support the carer to take regular breaks;
- g. Contacting an alternative care provider in advance to register with them in the event they are needed;
- h. Technology Enabled Care to increase security and safety at home;
- i. Increased monitoring of a Direct Payment.

Examples of reactive measures to risk that occur could include:

- a. Emergency care provision arranged by the Local Authority (the Local Authority has a duty to arrange alternative care provision in the event of provider failure);
- b. Changing from one care provider to another;
- c. Additional formal care in the absence of informal care;
- d. Requesting an urgent review of a Care and Support Plan;
- e. Another family member stepping in temporarily if carer breakdown occurs.

The contingency plan should be appropriate and proportionate based upon the level of risk identified and the likelihood of occurrence.

The contingency plan should be clearly recorded in both the draft and final Care and Support/Support Plan.

Any cost associated with contingency should be included in the indicative budget amount and built into the total cost of the Care and Support/Support Plan.

13. Deciding how to Manage the Personal Budget

13.1 Responsibilities and Requirements

One of the decisions that must be made when preparing the Care and Support Plan is how the personal budget will be managed.

You must provide information to the person/carers about the range of methods available to manage their personal budget so that they can make an

informed decision about which method would work best for them.

Wherever possible, the decision about how to best manage the personal budget should be made by the person/carer unless:

- a. They are a person with Care and Support needs and do not have the mental capacity to make this decision (in which case it must be made in their best interests);
- b. They request a Direct Payment but either do not meet the criteria or you deem it inappropriate for a Direct Payment to be provided; or
- c. The option requested is not available in the Local Authority.

Under the Care Act, a personal budget can be managed in 4 different ways:

- a. The Local Authority can manage the personal budget;
- b. The person can manage the personal budget as a Direct Payment; or
- c. A third party can manage the personal budget as an Individual Service Fund (ISF); or
- d. As a combination of these options.

Note: Not all areas provide the option to manage a personal budget as an Individual Service Fund (ISF). Before discussing ISF's with the person, you should familiarise yourself with local policy.

When deciding the best way for a personal budget to be managed you should consider:

- a. The thoughts, wishes and views of the person/carer;
- b. The availability of the services in the plan (to the person/carer and the Local Authority);
- c. The appropriateness of a Direct Payment;
- d. Where appropriate, the person's capacity to manage a Direct Payment; and
- e. Where the person lacks capacity, the availability of a suitable person to manage the Direct Payment.

You should explain to the person/carer that if the method of managing the personal budget does not prove to be the right method this can be changed at a later date.

13.2 Promoting Direct Payments

Specifically, the Local Authority is expected to promote the use of Direct Payments, as this provides the maximum level of choice and control. The ability to meet needs by taking a Direct Payment must be clearly explained to the person/carer, so that they can make an informed decision about whether they wish to take this option. This may mean offering the choice of a Direct Payment more than once during the planning process and enabling that choice by providing examples of how others have used direct payments, including via direct peer support from user-led organisations, for example.

See the [Direct Payment Procedure](#), including when a Direct Payment can and cannot be arranged and the process for doing so.

14. Deciding how the Plan will be Reviewed and Monitored

14.1 Statutory timescales for review

Under Section 27 of the Care Act 2014, the local authority **must** keep Care and Support/Support Plans under periodic review. This means they must have a system or process in place to ensure that reviews are carried out and monitored in a manner appropriate to the needs and circumstances of the person/carer whose plan it is.

Because every person/carer is different, the Care Act does not specify the frequency in which a Care and Support/Support Plan review must take place. That said, the statutory guidance does expect the local authority to endeavour to carry out a Care and Support/Support Plan review as follows:

- a. A review of a new service or Care and Support/Support Plan should be carried out within 6-8 weeks of the service/change commencing;
- b. A review of an on-going stable Care and Support/Support Plan should take place no less than 12 months after the date of the 6-8 week review, and then no less than once every 12 months after that;
- c. Where the person's/carer's needs or circumstances are likely to change reviews should be arranged as required to monitor the situation and respond to changes in a timely way (thus keeping the plan under review).

Note: If the person/carer is in receipt of a Direct Payment, statutory timeframes for review of the Direct Payment do apply. Wherever possible, Care and Support/Support Plan reviews should always be aligned with Direct Payment reviews to reduce duplication for both the person/carer and the

local authority.

For further guidance about Direct Payment reviews see: [Reviewing a Direct Payment](#).

14.2 Deciding the timeframe for review

Because the Care and Support/Support Plan is new, or is being revised the next review date should be no longer than 6-8 weeks' time. This should be clearly recorded on the Care and Support/Support Plan.

Where you are making arrangements for someone else to carry out the next review (rather than carrying it out yourself) you must make sure that you have recorded this in a way that will ensure the review takes place at the agreed time.

14.3 Information and advice about unplanned reviews

You must let the person/carer know how they (or anyone else) can request an unplanned review outside of the agreed timescale if:

- a. The Care and Support/Support Plan is not working as intended;
- b. Their needs change (increase or decrease);
- c. Their circumstances change;
- d. In the case of a person with Care and Support needs, the plan is affected by any change in the circumstances of an informal carer (for example if a carer is able to provide more or less support than anticipated in the plan).

14.4 Monitoring

The Care Act requires you to make proportionate and appropriate arrangements to monitor the Care and Support/Support Plan whenever this is

required (thus keeping the plan under review).

Examples of situations when monitoring may be required include:

- a. If the plan is deemed unstable even with contingency;
- b. If the person's/carer's needs are expected to change in the short term (impacting on the level of support they will need);
- c. If there is a likely risk of deterioration in needs or circumstance without monitoring;
- d. As part of ongoing MDT involvement;
- e. If you are co-ordinating the follow up of an action plan agreed at review;
- f. To manage the risk from abuse or neglect.

The arrangements to monitor must be:

- a. Proportionate and appropriate to the person's situation and risk;
- b. Agreed with the person/carer's; and
- c. Agreed from a strengths based approach to promote resilience of the person/carer and their informal networks of support.

If you are unclear about the need to monitor you should seek advice from your line manager.

Need to Know

On-going monitoring can increase the likelihood of dependency on the Local Authority and it is important to build resilience wherever possible.

To reduce the risk of dependency decisions about monitoring must be made taking into account evidence that confirms the actual level of risk in not doing so (as opposed to the perceived risk that may be associated with anxiety only).

Monitoring arrangements must be clearly recorded. Where you are making arrangements for someone else to monitor the situation (rather than monitoring it yourself) you must make sure that you have recorded this in a way that will ensure the monitoring activity takes place at the agreed

time.

Any monitoring activity carried out should be clearly recorded and you should use supervision effectively to discuss and agree the need for continued monitoring.

15. Safeguarding and Deprivation of Liberty

15.1 Safeguarding

If, as part of any conversation you have with a person/carer or their family you become concerned that a vulnerable adult or a child is experiencing, or at risk of abuse or neglect you **must** respond appropriately.

See [Safeguarding Adults](#), which also includes information about how to raise a children's safeguarding concern.

If you are concerned that an adult or child is in imminent danger from abuse or neglect, or that a criminal act has taken place you should contact the police by dialling 999.

Where the safeguarding is in respect of the person/carer whose Care and Support/Support Plan is being developed a decision will need to be made about the need to pause the process to allow a safeguarding enquiry to take place.

There are 3 possible options:

- a. The Care and Support/Support planning process continues alongside any safeguarding process;
- b. The Care and Support/Support planning process is paused with no on-going intervention by the person carrying it out whilst a safeguarding process takes place; or
- c. The Care and Support/Support planning process is paused but urgent interim support is arranged to ensure needs are met whilst a safeguarding process takes place.

Any decision should involve the person carrying out the Care and Support planning process, the person who will be carrying out any safeguarding process, the person with Care and Support needs (or their representative) / carer.

15.2 Deprivation of Liberty

You must consider any appropriate action required to authorise deprivations of liberty whenever:

- a. A person with Care and Support needs lacks capacity to make decisions about the Care and Support provided to them; and
- b. You feel the level of restriction being imposed on the person is depriving them of their liberty; or
- c. You feel the level of restriction required to meet their care and support needs following assessment is likely to deprive them of their liberty.

See: [Recognising and Responding to Deprivations of Liberty](#).

16. Combining Care and Support Plans with Other Plans

16.1 Another plan for the same person

If the person with Care and Support needs has more than one plan of any kind it may be appropriate to combine the plans into one so as to:

- a. Prevent duplication of information;
- b. Prevent the person from having to take part in more than one planning process;
- c. Reduce administration.

Plans can only be combined if:

- a. The person is in agreement (or a decision is made in their best interests if they lack capacity); and
- b. Combining plans is technologically possible.

It is your responsibility to seek clarity about which plans can and cannot be combined before suggesting this as an option for the person.

Even if it is not possible to record separate plans on the system efforts should be made to carry out a single planning process with the person in which to gather all of the information required for all of the plans at the same time.

This may require joint working with another service area or team or with another organisation (for example health or housing).

Under the Care Act you are able to request the co-operation of any other organisation or person and that person or organisation has a duty to co-operate with the request unless doing so would:

- a. Prevent them from carrying out their own statutory duties under the Care Act or other legislation;
- b. Have an adverse effect on their general functions.

See: [Co-Operation](#) to read more about co-operation under the Care Act.

16.2 A Carers Support Plan

When the person with Care and Support needs has a carer it may be beneficial to combine the Care and Support Plan with the Support Plan for the carer.

Plans can only be combined if:

- a. The person is in agreement (or a decision is made in their best interests if they lack capacity);
- b. The carer is in agreement; and
- c. Combining plans is technologically possible.

It is your responsibility to seek clarity about which plans can and cannot be combined before suggesting this as an option for the person and carer.

Even if it is not possible to record separate plans on the system efforts should be made to carry out a single planning process with the person and carer in which to gather all of the information required for all of the plans at the same time.

17. Equipment and Adaptations

Any equipment or adaptations that are provided to meet an eligible need should be included in the Care and Support Plan.

It is the role of the practitioner who arranged for the equipment or adaptation to provide the following information to the practitioner or service with statutory responsibility for reviewing and maintaining the Care and Support Plan:

- a. The equipment or adaptation that has been provided;
- b. The date that the equipment or adaptation was provided; and
- c. The cost of any equipment over £1000 (where the local policy is to charge a person for equipment costing more than £1000).

You should then take steps to include the equipment or adaptation in the Care and Support Plan.

The cost of the adaptation or equipment should be recorded as zero unless:

- a. The cost of any equipment is over £1000; and
- b. The local policy is to charge for equipment over £1000.

18. Recording a Care and Support/Support Plan

Note: You are responsible for establishing the current framework used by the Local Authority for recording a Care and Support/Support Plan. If you are unclear you should speak to your line manager before proceeding to make a formal record of the plan.

18.1 The timeframe for making a record

Draft Care and Support/Support Plans should be recorded in a timely way and in line with local requirements.

Timely recording will:

- a. Reduce the likelihood of inaccuracies;
- b. Prevent any unnecessary delays for the person/carer; and

- c. Ensure that the duty to meet eligible needs is met as close to the need being identified as possible.

18.2 Information gathered through the process

To ensure the involvement of the person/carer and to ensure that the Care and Support planning process has been carried out in an accessible way there will be at least one (but possibly a range) of informal records made. Examples could include:

- a. A blank copy of the Local Authority template with handwritten notes;
- b. A tool that has been used by the person/carer to explore Wellbeing;
- c. Notes made about the different available services and associated costs;
- d. A pros and cons list to support decision making about the best way to meet eligible needs from the options available;
- e. A record of risks used for contingency planning.

It is important that you use all of the information gathered and record it on the Care and Support/Support Plan where it is relevant to do so.

Any informal records should then be filed, destroyed securely or returned to the person/carer (if they have requested this) with full regard for confidentiality.

18.3 The information that must be included

The following information must be recorded on the draft plan before it can be signed off:

- a. The needs identified by the assessment;
- b. Which of these needs meet the national eligibility criteria;
- c. Which needs the Local Authority is going to meet and how it is going to meet them;
- d. How any risk to the plan will be managed;

- e. Which needs are going to be met informally, or through alternative services;
- f. The information and advice that has been given about reducing, delaying or preventing needs that the Local Authority is not going to meet;
- g. How you propose the plan will be reviewed or monitored;
- h. The method of managing the personal budget proposed;
- i. Where a Direct Payment is proposed, which needs are to be met with the Direct Payment;
- j. Where a Direct Payment is proposed, any associated employer related costs;
- k. The indicative personal budget; and
- l. The proposed final personal budget.

18.4 Recording respite and other services to support a carer

When services to support a carer are being provided directly to the person with Care and Support needs (for example respite services) it is important that the services are recorded appropriately to ensure that the carer is not charged for the services:

- a. The description of the service being provided should be recorded on both the Care and Support Plan and the carer's Support Plan (as part of the contingency plan); but
- b. The cost of the service should only be recorded on the Care and Support Plan (as the service is not being provided to the carer as part of their personal budget).

18.5 When the cost of the plan is more than the proposed personal budget

Sometimes the cost of the total plan will exceed the amount of the personal budget being requested. This is because the Care and Support/Support Plan may detail all of the ways that a person's/carer's needs are to be met, not just those methods being provided by the Local Authority. For example:

- a. The person/carer wants to include a social activity in their plan; or

b. In the case of a Care and Support Plan, the local ICB have agreed to contribute some of the cost of the plan because of the person's level of health needs (known as joint funding).

Where there is a difference between the total cost of the plan and the personal budget, the amount that anyone else is going to pay should be clearly recorded on the plan.

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